

1. Administrative & Study Identification

Full Study Title: A Study Comparing Savolitinib Plus Osimertinib vs Savolitinib Plus Placebo in Patients With EGFRm+ and MET Amplified Advanced NSCLC **Short Title/Acronym:** CoC Study (Contribution of Components) **Protocol Number:** D5084C00009 **NCT Number:** NCT04606771 **Posting ID:** 339789470926583693 **Trial Status:** ACTIVE_NOT_RECRUITING **Sponsor Name:** AstraZeneca (Company: AstraZeneca) **Investigational Product:** Savolitinib + osimertinib (Trade Name: Tagrisso) [ATC Code: L01EB04] **Phase of Development:** PHASE2 **Medical Monitor:** AstraZeneca Medical Monitor

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the Objective Response Rate (ORR) per RECIST 1.1 (investigator-assessed) of savolitinib plus osimertinib compared with savolitinib plus placebo. **Secondary Objectives:** To assess Progression-Free Survival (PFS), Duration of Response (DOR), Tumour Size Assessment (TSA), Overall Survival (OS), Pharmacokinetics (PK), and Safety/Tolerability.

2.2 Background & Rationale Osimertinib (an EGFR-TKI) is the preferred first-line treatment for Epidermal Growth Factor Receptor mutated (EGFRm+) advanced Non-Small Cell Lung Cancer (NSCLC); however, resistance eventually develops. Amplification of the MET receptor tyrosine kinase is the most common known resistance mechanism, which activates downstream intracellular signaling independent of EGFR. Combining osimertinib with savolitinib (a MET-TKI) has shown promising antitumor activity. This study aims to assess the individual contribution of the components to the combination regimen by comparing the combination to a savolitinib + placebo control arm in patients who have progressed on prior osimertinib.

3. Study Design & Methodology

3.1 Design Framework Study Type: INTERVENTIONAL **Control Type:** Placebo-controlled (Active targeted therapy + Placebo) **Blinding:** Double-blind **Randomization:** 1:1, stratified by the number of prior lines of therapy

3.2 Planned Enrollment & Duration Targeted Enrollment: 30 patients **Number of Sites:** Multicenter **Estimated Study Start:** 28-Sep-2020 **Estimated Primary Completion:** 21-Dec-2022

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Patients must be ≥ 18 years of age (≥ 20 years in Japan). 2. Histologically or cytologically confirmed locally advanced or metastatic EGFRm+ NSCLC (e.g., exon 19 deletion and/or L858R) not amenable to curative therapy. 3. Documented radiologic disease progression following treatment with osimertinib. 4. MET-amplified disease, centrally confirmed by FISH (MET gene copy ≥ 5 or MET/CEP7 ratio ≥ 2).

4.2 Exclusion Criteria 1. Prior or current treatment with a 3rd generation EGFR-TKI other than osimertinib. 2. Prior or current treatment with savolitinib or another MET inhibitor (e.g., crizotinib, capmatinib). 3. Any cytotoxic chemotherapy or other anti-cancer drugs within 14 days prior to the first dose. 4. Past medical history of interstitial lung disease (ILD), drug-induced ILD, or radiation pneumonitis requiring steroid treatment.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * Arm A: Savolitinib 300 mg QD + osimertinib 80 mg QD. * Arm B: Savolitinib 300 mg QD + Placebo QD. Route of Administration: Oral Duration of Treatment: Administered in 28-day cycles until objective disease progression (per RECIST 1.1), unacceptable toxicity, or consent withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care measures. Prohibited: Concomitant use of any medications, herbal supplements, or foods with known strong inducer effects on CYP3A4 during the study and for 3 months after the last dose.

6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures | | :--- | :--- | :--- | | :--- | :--- | | Screening | Day -28 to 0 | Informed Consent, Medical History, Tumor biopsy for central MET amplification (FISH) assessment | | Baseline | Day 1 | Randomization, First Dose | | Interim | Every 28 Days | Safety Labs, Efficacy Assessment (Tumor imaging), PK sampling, AE monitoring | | End of Study | Variable | Disease progression, Exit Interview, IP Accountability, Survival follow-up |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Objective Response Rate (ORR), defined as the percentage of evaluable patients with an Investigator-assessed visit response of Complete Response (CR) or

Partial Response (PR). **Measurement Method:** Investigator-assessed per RECIST v1.1.

7.2 Secondary & Exploratory Endpoints * Progression-Free Survival (PFS) * Duration of Response (DoR) * Tumour Size Assessment (TSA) * Overall Survival (OS) * Pharmacokinetic parameters and Safety/Tolerability

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via regular site visits and clinical laboratory evaluations, monitoring specifically for known AEs of the TKIs (e.g., nausea, peripheral edema, vomiting). **Serious Adverse Events (SAEs):** Continuously monitored and reported within standard 24-hour timelines to the sponsor. **Data Monitoring Committee (DMC):** Internal safety review mechanisms managed by AstraZeneca.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) Population for primary efficacy analysis; Safety Analysis Set for all AE/SAE evaluations. **Handling of Missing Data:** Standard survival analysis methodologies (e.g., censoring for unobserved events in PFS and OS).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring visits to ensure protocol adherence. **Audit Trail:** Data entered into an eCRF with locking mechanisms, query resolution, and central laboratory confirmation of MET amplification status.

11. Study Identifiers & Governance

Primary ID: D5084C00009 **Registry Number:** NCT04606771 **Posting ID:** 339789470926583693 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** A Study Comparing Savolitinib Plus Osimertinib vs Savolitinib Plus Placebo in Patients With EGFRm+ and MET Amplified Advanced NSCLC **Official Regulatory Title:** A Multi-centre Phase II, Double-Blind, Randomised Study of Savolitinib in Combination With Osimertinib vs Savolitinib in Combination With Placebo in Patients With EGFRm+ and MET Amplified Locally Advanced or Metastatic Non-Small Cell Lung Cancer Who Have Progressed Following Treatment With Osimertinib **EMA URL:** <https://>

12. Clinical Framework (Oncology / NSCLC Specific)

Primary Condition / Disease Area: Non-Small Cell Lung Cancer (NSCLC) progressing after osimertinib **Preferred UMLS Name:** Advanced Non-Small Cell Lung Carcinoma with EGFR Mutation and MET Amplification **Diagnosis Threshold:** EGFRm+ and MET-amplified (MET gene copy ≥ 5 or MET/CEP7 ratio ≥ 2) **Medical Methodology:** Targeted Tyrosine Kinase Inhibitor (TKI) Therapy **Optimization Target:** Overcoming MET-driven secondary resistance **MeSH Code:** D016938 **SNOMED ID:** 457721000124104

13. Study Procedures & Methodology

Management Pathway: Resistance assessment and combinational targeted therapy **Education Protocol (HCP):** Protocol-specific training including management of TKI-related toxicities (e.g., ILD, QTc monitoring) **Education Protocol (Patient):** Informed consent, daily oral dosing compliance, and AE self-reporting **Quality Audit Mechanism:** Central imaging review and independent central FISH testing for biomarker eligibility

14. Observation & Follow-Up Schedule

Total Duration: Treatment until objective disease progression or unacceptable toxicity **Onsite Visit Cadence:** Every 28-day treatment cycle **Remote Monitoring Frequency:** As indicated for safety follow-up **Checklist Review Interval:** Regular tumor assessment intervals (e.g., every 6-8 weeks) per RECIST 1.1

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	____	[DD-MMM-YYYY]				